CHAPTER H-05

HEALTH INSURANCE CLAIMS

Chapter Introduction

In this chapter we will discuss about claim management process in Health Insurance, claims related procedures and documentation. Apart from this, we will also look into claims management under Personal Accident Insurance and understand the role of TPAs.

Learning Outcomes

A. Claims Management in insurance

B. Management of Health Insurance claims

C. Documentation in Health Insurance claims

D. Role of Third Party Administrators (TPA)

E. Claims Management – Personal Accident

F. Claims Management- Overseas Travel Insurance

After studying this chapter, you should be able to:

a) Explain the various stakeholders in insurance claims
b) Describe how health insurance claims are managed
c) Discuss the various documents required for settlement of health insurance claims
d) Explain how reserves for claims are provided for by insurers.
e) Discuss personal accident claims
f) Understand the concept and role of TPAs

A. Claims Management in Insurance

It is very well understood that insurance is a '**promise**' and the policy is a '**witness**' to that promise. The occurrence of an insured event leading to a claim under the policy is the true test of that promise. How well an insurer performs is evaluated by how well it keeps its claims promises. One of the key rating factors in insurance is the claims paying ability of the insurance company.

1. Stakeholders in claim process

One needs to understand the parties interested in the claims process before looking at how claims are managed.

Diagram 1: Stakeholders in claim process

Customer	The person who buys insurance is the first stakeholder and 'receiver of the claim'.
Owners	Owners of the insurance company have a big stake as the 'payers of the claims'. Even if the claims are met from the policy holders' funds, in most cases, it is they who are liable to keep the promise.
Underwriters	Underwriters within an insurance company and across all insurers have the responsibility to understand the claims and design the products, decide policy terms, conditions and pricing etc.
Regulator	The regulator (Insurance Regulatory and Development Authority of India) is a key stakeholder in its objective to: ✓ Maintain order in the insurance environment ✓ Protect policy holders' interest ✓ Ensure long term financial health of insurers.

Third Party Administrators	Service intermediaries known as Third Party Administrators, who process health insurance claims.
Insurance agents/ brokers	Insurance agents/ brokers not only sell policies but are also expected to service the customers in the event of a claim.
Providers/ Hospitals	They ensure that the customer gets a smooth claim experience, especially when the hospital is on the panel of the TPA the Insurer to provide cashless hospitalization.

Thus managing claims well means managing the objectives of the each of these stakeholders related to the claims. Of course, it may happen that some of these objectives can conflict with each other.

Reserving: In many cases, insurance companies may not be able to settle claims instantly and may have to wait for information or the results of disputes, litigation etc. So, they have to hold the claim amounts in reserve till the payments are due. Reserves are usually are actuarial estimates of the amounts that will be paid on outstanding claims.

Reserving refers to the amount of provision made for all claims in the books of the insurer based on the status of the claims.

Test Yourself 1

Who among the following is not a stakeholder in Health insurance claim process?

I. Customers
II. Police Department
III. Regulator
IV. TPA

B. Management of Health Insurance Claims

1. Claim process in health insurance

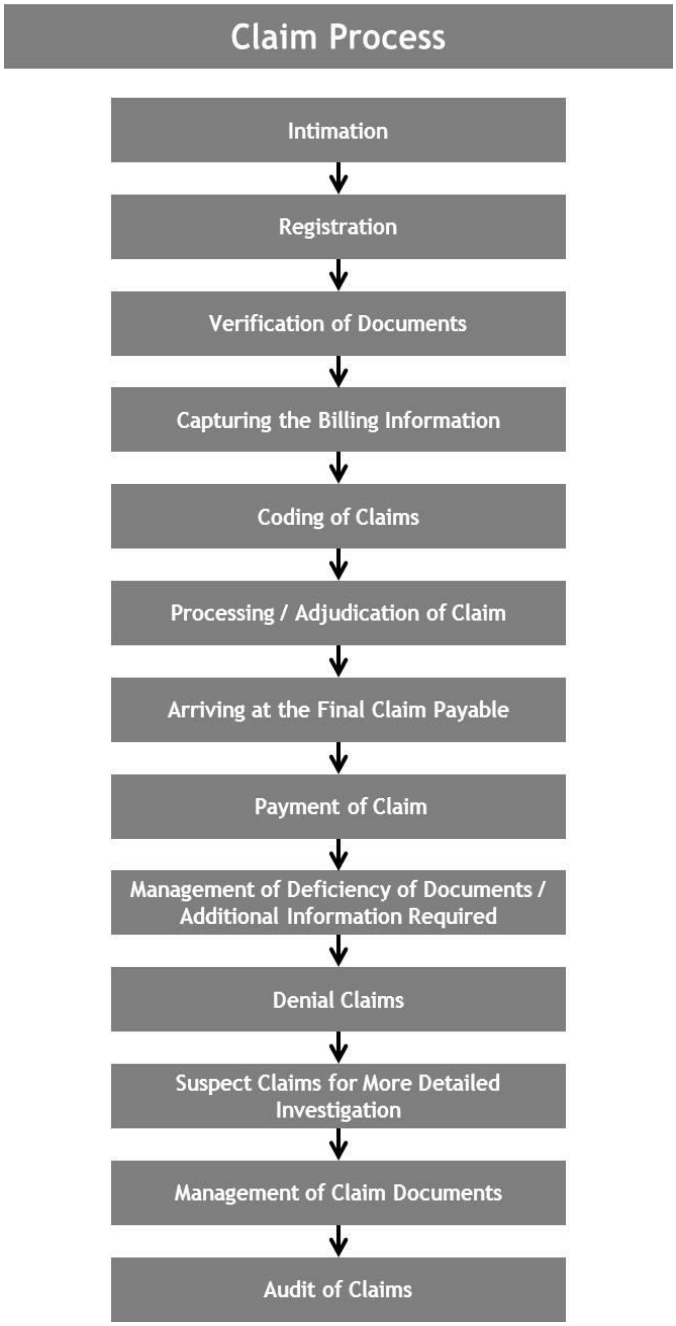
A claim may be serviced either by the insurance company itself or through the services of a Third Party Administrator (TPA) authorized by the insurance company.

From the time a claim is made known to the insurer/ TPA to the time the payment is made as per the policy terms, the health claim passes through a set of well-defined steps, each having its own relevance.

The processes detailed below are in specific reference to health insurance (hospitalization) indemnity products which form the major part of health insurance business.
The general process and supporting documents for a claim under fixed benefit product or critical illness or daily cash product etc. would be quite similar, except for the fact that such products may not come with cashless facility.

In both cases of indemnity as well as reimbursement type of claim, the basic steps remain the same.

Diagram 2: Claim process broadly comprises following steps (may not be in the same order)



a) Intimation

Claim intimation is the first instance of contact between the customer and the claims team. The customer could inform the company that he is planning to avail a hospitalization or the intimation would be made after the hospitalization has taken place, especially in case of emergency admission to a hospital.

Till recently, the act of intimation of a claim event was a formality. However, recently insurers have started insisting on the intimation of claim as soon as practicable. Typically it is required before hospitalization in case of planned admission, and within 24 hours of hospitalization in case of an emergency. Intimation is now possible through Mobile Apps/ call centres run by insurers/ TPAs open 24 hours as well as through the internet and e-mail.

b) Registration

Once the intimation is received by the company directly or through the TPA, the details thereof are matched for accuracy and a reference number or claim control number generated and intimated to the claimant. The documents are then scrutinized for prima facie coverage and pre-authorisation of likely expenditure is given to the Hospital in case the intimation is of a planned surgery under the Cash-less scheme (detailed in subsequent section).

The claims that come for the final settlement on the reimbursement basis are scrutinized in detail about admissibility, sum assured, deductibles, sub-limits etc. In case of deficiency in documents the same has to be communicated together, not in piecemeal. It is worth knowing that the claim processing involves not only ensuring that the terms of the contract have to be fulfilled, but also in ensuring that the Hospitals do not indulge in overcharging, double-charging etc.

Example

Hospitalization is typically associated with Allopathic method of treatment. However, the patient could undergo other modes of treatment such as:

- ✓ Unani
- ✓ Siddha
- ✓ Homeopathy
- ✓ Ayurveda
- ✓ Naturopathy etc.

Most policies now include these treatments, however there could be sub-limits.

Telemedicine: IRDAI has asked insurers to allow telemedicine wherever regular medical consultation is allowed, in the terms and conditions of medical insurance policies.

This will help policy holders who may prefer to consult medical practitioners online or telephonically to avoid going out of their homes or if they are in quarantine themselves due to the coronavirus infection.

Arriving at the final claim payable: The factors that decide the claim amount payable are:

a) Sum insured available for the member under the policy

b) Balance sum insured available under the policy for the member after taking into account any claim made already:

c) Sub-Limits

d) Check for any limits specific to illness

e) Check whether entitled or not to cumulative bonus

f) Other expenses covered with limitation:

What are finally paid are the Reasonable and Customary Charges meaning the charges for services or supplies, which are the standard charges for the specific provider and consistent with the prevailing charges in the geographical area for identical or similar services, taking into account the nature of the illness/ injury involved.

Earlier every TPA/ insurer had its own list of non-payable items, now the same has been standardized under IRDAI Health Insurance Standardization Guidelines.

c) Payment of claim

Once the payable claim amount is arrived at, payment is done to the customer or the hospital as the case may be. The payment may be made either by cheque or by transferring the claim money to the customer's bank account.

d) Denial of claims

The experience in health claims show that 10% to 15% of the claims submitted do not fall within the terms of the policy. This could be because of a variety of reasons some of which are:

i. Date of admission is not within the period of insurance.
ii. The Member for whom the claim is made is not covered.
iii. Due to Pre-existing illness (where the policy excludes such condition).
iv. Undue delay in submission without valid reason.
v. No active treatment; admission is only for investigation purpose.
vi. Illness treated is excluded under the policy.
vii. The cause of illness is abuse of alcohol or drugs
viii. Hospitalization is less than 24 hours.

Denial or repudiation of a claim (due to whatever reason) has to be informed to the customer in writing by the insurance company. Usually, such denial letter clearly states the reason for denial, narrating the policy term/ condition on which the claim was denied.

Apart from the representation to the insurer, the customer has the option to approach the following in case of denial of claim:

- ✓ Insurance Ombudsman or
- ✓ The Consumer Commissions or
- ✓ IRDAI or
- ✓ Law courts.

e) Suspect claims require more detailed investigation by the companies/ TPAs

Wherever the insurance company suspects foul-play it can get claims investigated. A few examples of frauds committed in health insurance are:

i. Impersonation, the person insured is different from person treated.
ii. Fabrication of documents to make a claim where there is no hospitalization.
iii. Inflation of expenses, either with the help of the hospital or by addition of external bills fraudulently created.
iv. Outpatient treatment converted to in-patient/ hospitalization to cover cost of diagnosis, which could be high in some conditions.

It is to be noted that in respect of claims that need to be investigated, investigations shall be initiated and completed at the earliest, in any case not later than 90 days from the date of receipt of claim intimation. The claim should be settled within 30 days of completing the investigation. (Pl refer to IRDAI (Protection of policyholder's), 2017 Regulations and updated accordingly)

f) Cashless settlement process by TPA

How does the cashless facility work? At the heart of this is an agreement that the TPA insurer enters into, with the hospital. There are agreements possible with other medical service providers as well. The process used for providing cashless facility are discussed in this section:

Table 3.1

Step 1	• A customer covered under health insurance suffers from an illness or sustains an injury and so is advised admission into a hospital. He/ she (or someone on his/ her behalf) approaches the hospital's insurance desk with the insurance details such as: i. TPA name, ii. Customer's membership number, iii. Insurer's name, etc.
Step 2	• The hospital compiles the necessary information such as: i. Diagnosis of illness ii. Treatment, iii. Name of treating doctor, iv. Number of days of proposed hospitalization and v. The estimated cost • This is presented in a format, called the cashless authorization form.
Step 3	• The TPA studies the information provided in the *cashless authorization form* and takes a decision on whether the cashless authorization could be provided and if so, for how much amount it should be authorized and it is communicated to the hospital without delay.
Step 4	• The patient is treated by the hospital, keeping the amount authorized by the TPA as credit in the patient's account. The member may be called on to make a deposit payment to cover the non-treatment expenses and any co-pay required under the policy.
Step 5	• When the patient is ready for discharge, the hospital checks the amount of credit in the account of the patient approved by the TPA against the actual treatment charges covered by insurance. • If the credit is less, the hospital requests for additional approval of credit for the cashless treatment. • TPA analyses the same and approves the additional amount.
Step 6	• Patient pays the non-admissible charges and gets discharged. He will be asked to sign the claim form and the bill, to complete the documentation.

Step 7	• Hospital consolidates all the documents and presents to the TPA the documents for processing of the bill
Step 8	• TPA will process the claim and recommend for payment to the hospital after verifying details.

g) Customer must make sure that he/ she has his/ her insurance details with him/ her.

This includes his TPA card, Policy copy, Terms and conditions of cover etc.

When these are not available, he can contact the TPA (through a 24 hour helpline) and seek the details.

i. Customer must check if the hospital suggested by his/ her consulting doctor is in the network of the TPA. If not, he needs to check with the TPA the options available where cashless facility for such treatment is available.

ii. He/ she needs to make sure that the correct details are entered into the pre-authorization form. This form has been standardized by IRDAI as per Guidelines on Standardization in Health Insurance issued in 2013. If the case is not clear, the TPA could deny the cashless facility or raise query.

iii. He/ she needs to ensure that the hospital charges are consistent with the limits such as room rent or caps on specified treatments such as cataract.

iv. The customer must inform the TPA in advance of the discharge and request the hospital to send to the TPA any additional approval that may be required before discharge. This will ensure the patient does not wait unnecessarily at the hospital.

It is also possible that the customer requests and takes an approval for cashless treatment at a hospital but decides to admit the patient elsewhere. In such cases, the customer must inform and ask the hospital to communicate to the TPA that the cashless approval is not being used.

If this is not done, the amount approved could get blocked in the customer's policy and could prejudice the approval of the subsequent request.

C. Documentation in Health Insurance Claims

This section explains the need for and content of each of the documents required to be submitted by the customers:

1. Discharge summary

Discharge summary can be termed as the most important document that is required to process a health insurance claim. It details the complete information about the condition of the patient and the line of treatment and helps the claim processing person immensely to understand the illness/ injury and the line of treatment. Where the patient unfortunately does not survive, the discharge summary is termed **Death Summary** in many hospitals. The discharge summary is always sought in original.

2. Investigation reports

Investigation reports assist in comparing the diagnosis and the treatment, thereby providing the necessary information to understand the exact condition that

prompted the treatment and the progress made during the hospitalization for e.g. Blood test reports, X-ray reports and Biopsy reports. The insurer may return the X-ray and other films to the customer on specific request.

3. Consolidated and detailed bills:

This is the document that decides what needs to be paid under the insurance policy. While the consolidated bill presents the overall picture, the detailed bill will provide the break up, with reference codes. The bills have to be received in original.

4. Receipt for payment

The reimbursement of a health insurance claim will also require the formal receipt from the hospital of the amount paid which must correspond to the total of the bill.

The receipt should be numbered and or stamped and be presented in original.

5. Claim form

Claim form is the formal and legal request for processing the claim and is submitted in original signed by the customer. The claim form has now been standardized by IRDAI.

Besides information on disease, treatment etc., the declaration the insured person makes in the claim form is the most important document in the legal sense.

6. Identity proof

With the increasing use of identity proof across various activities in our life, the general Proof of identity helps in verifying whether the person covered and the person treated are one and the same. Usually identification document which is sought could be voters' identity card, driving license, PAN card, Aadhaar card etc.

7. Documents contingent to specific claims

There are certain types of claims that require additional documents apart from what has been stated above. These are:

a) Accident claims, where FIR or Medico-legal certificate issued by the hospital to the registered police station, may be required.

b) Case indoor papers in case of complicated or high value claims.

c) Dialysis/ Chemotherapy/ Physiotherapy charts where applicable.

d) Hospital registration certificate, where the compliance with the definition of hospital needs to be checked

Test Yourself 2

Which of the following document is maintained at the hospital detailing all treatment done to an in-patient?

I. Investigation report

II. Discharge summary

III. Case paper

IV. Hospital registration certificate

Test Yourself 3

The amount of provision made for all claims in the books of the insurer based on the status of the claims is known as _____.

I. Pooling
II. Accounting
III. Reserving
IV. Investing

D. Role of Third Party Administrators (TPA)

The Role of TPA has been discussed in earlier chapters too. It is important to know the services offered by TPA so that the customer can be provided suitable services by the salesperson.

The scope of TPA services starts after the sale and issue of the insurance policy. In case of insurers not using TPAs, the services are performed by in-house team.

1. Post sale service of health insurance

a) Once the proposal (and the premium) is accepted, the coverage commences.
b) If a TPA is to be used for servicing the policy, the insurer passes on the information about the customer and the policy to the TPA.
c) The TPA enrols the members (while the proposer is the person taking the policy, members are those covered under the policy) and may issue a membership identification in the form of a card, either physical or electronic.
d) The membership with the TPA is used for availing cashless facility as well as processing of claims when the member requires the support of the policy for a hospitalization or treatment that is covered.
e) TPA processes the claim or cashless request and provides the services within the time agreed with the insurer.
f) The insured persons must carry an Identity Card that relates them to the policy and the TPA.
g) TPA issues a pre-authorization or a Letter of Guarantee to the hospital based on the information provided for requesting the cashless facility.
h) Where the information is not clear or not available, the TPA may reject the cashless request. In such cases the claim could be examined on reimbursement basis.

2. Customer relationship and contact management

Since TPAs are involved in claims servicing, they usually have a grievance redressal mechanism themselves.

E. Claims Management – Personal Accident

On receipt of the notification of the claim the following aspects should be looked into:

a) Person in respect of whom the claim is made is covered under the policy
b) Policy is valid as on date of accident and premium has been received
c) Loss is within the policy period

d) Loss has arisen out of “Accident” and not sickness
e) Check for any fraud triggers and assign investigation if need be
f) Register the claim and create reserve for the same
g) Maintain the turnaround time (claim servicing time) and keep the customer informed of the development of the claim.

1. Claims Investigation

Claims Investigation is about determining the validity of the claim and finding out the real cause and extent of the loss. On receipt of the claim documents, if a claim appears suspicious, the claim may be assigned to an internal/ professional investigator for verification.

Example

Example of case guideline:
Road traffic accident

i. When did the incident take place – exact time and date place? Date and time
ii. Was the insured a pedestrian, traveling as passenger/ pillion rider or driving the vehicle involved in accident?

Some examples of possible fraud and leakage in personal accident claims:

i. Exaggeration in TTD period.
ii. Illness presented as accident e.g. backache due to pathological reasons converted into a PA claim after reported ‘fall/ slip’ at home

Discharge voucher is an important document for settlement of personal accident claim, especially those involving death claims. It is also important to obtain nominee details at the time of proposal and the same should form part of policy document.

2. Claim documentation- Each company gives a list

a) Duly completed Personal Accident claim form signed by the claimant’s nominee/ family member
b) Original or Attested copy of First Information Report.
c) Original or Attested copy of Death certificate.
d) Attested copy of Post Mortem Report if conducted.
e) Attested copy of AML documents (Anti-money laundering) - for name verification (passport/ PAN card/ Voter's ID/ Driving license) for address verification (Telephone bill/ Bank account statement, Electricity bill/ Ration card).
f) Legal heir certificate containing affidavit and indemnity bond both duly signed by all legal heirs and notarized
g) Permanent disability certificate from a civil surgeon or any equivalent competent doctors certifying the disability of the insured.
h) Medical certificate from treating doctor mentioning the type of disability and disability period. Leave certificate from employer giving details of exact leave period, duly signed and sealed by the employer.

The above list is only indicative, further documents (including photographs of scar marks, site of accident etc.) may be required depending on particular facts of the case, especially the cases with suspected fraud angle to be investigated.

Test Yourself 4

Which of the following documents are not required to be submitted for Permanent Total Disability claim?

I. Duly completed Personal Accident claim form signed by the claimant.
II. Copy of Insurance Policy.
III. Permanent disability certificate from a civil surgeon or any equivalent competent doctors certifying the disability of the insured.
IV. Fitness certificate from the treating doctor certifying that the insured is fit to perform his normal duties.

F. Claims Management- Overseas Travel Insurance

The coverage under this policy has already been discussed under the product chapter. This section tries to explain how the claims arising during overseas travel are handled.

Claims services essentially include:

a) Taking down the claim notification 24*7 basis;
b) Sending the claim form and procedure;
c) Guiding customer on what to do immediately after loss;
d) Extending cashless services for medical and sickness claims;
e) Arranging for repatriation and evacuation, emergency cash advance.

Assistance companies – Role in overseas claims

Assistance companies have their own offices and tie up arrangements with other similar service providers world over. These companies offer assistance to the customers of insurance companies in case of contingencies covered under the policy.

These companies operate a 24*7 call centre including international toll free numbers for claim registration and information. They also offer the following services and charges for the services vary depending on agreement with the particular insurance company, benefits covered etc.

a) Medical assistance services:
   i. Medical service provider referrals
   ii. Arrangement of hospital admission
   iii. Arrangement of Emergency Medical Evacuation
   iv. Arrangement of Emergency Medical Repatriation
   v. Mortal remains repatriation
   vi. Compassionate visit arrangements
   vii. Minor children assistance/ escort
b) Monitoring of Medical Condition during and after hospitalisation
c) Delivery of Essential Medicines
d) Guarantee of Medical Expenses Incurred during hospitalization subject to terms and condition of the policy and approval of insurance company.
e) Pre-trip information services and other services:
   i. Visas and inoculation requirements
   ii. Embassy referral services
   iii. Lost passport and lost luggage assistance services
   iv. Emergency message transmission services
   v. Bail bond arrangement
   vi. Financial Emergency Assistance

f) Interpreter Referral
g) Legal Referral
h) Appointment with lawyer

a) Hospitalization Procedures

i. Most hospitals accept Guarantee of Payments from all international insurance companies once the insured provides them with a valid health or overseas travel insurance policy.

ii. Hospitals start the treatment immediately. If there is insurance cover the insurance policy pays or the patient person has to pay. The hospitals tend to inflate charges since payments are delayed.

iii. Information regarding network hospitals and the procedures is available to the insured on the toll free numbers provided by the assistance companies.

iv. In event of the necessity of a hospitalization the insured needs to intimate the same at the call centre and proceed to a specified hospital with the valid travel insurance policy.

v. Hospitals usually contact the assistance companies/ insurers on the call centre numbers to check the validity of the policy and verify coverages.

vi. Once the policy is accepted by the hospital the insured would undergo treatment in the hospital on a cashless basis.

vii. Some basic information required by the insurer/ assistance provider to determine admissibility are:

1. Details of ailment
2. In case of any previous history ,details of hospital, local medical officer in India:
   - ✓ Past history, current treatment and further planned course in hospital and request for immediate sending of
   - ✓ Claim form along with attending physicians statement
   - ✓ Passport copy
   - ✓ Release of medical information form

b) Reimbursement of medical expenses and other non-medical claims:

Reimbursement claims are normally filed by insured after they return to India. Upon receipt of the claim papers, claim is processed as per usual process. Payments for all admissible claims are made in Indian Rupee (INR), unlike in cashless claims where payment is made in foreign currency.

While processing the reimbursement claims, currency conversion rate is applied as on date of loss to arrive at quantum of liability in INR. Then the payment is made though cheque or electronic transfer.

c) Claim documentation for Medical Accident and Sickness Expenses

i. Claim form

ii. Doctor’s report

iii. Original Admission/ discharge card

iv. Original Bills/ Receipts/ Prescription

v. Original X-ray reports/ Pathological/ Investigative reports

vi. Copy of passport/ Visa with Entry and exit stamp

The above list is only indicative. Additional information/ documents may be required depending on specific case details or depending upon claim settlement policy/ procedure followed by particular insurer.

Test Yourself 5

Most hospitals accept Guarantee of Payments from all international insurance companies once the insured provides them with a valid _____ Insurance policy.

I. Legal Liability
II. Corona Rakshak
III. Overseas Travel
IV. Endowment

Answers to Test Yourself

Answer 1 - The correct option is II.
Answer 2 - The correct option is II.
Answer 3 - The correct option is III.
Answer 4 - The correct option is IV.
Answer 5 - The correct option is III.

Summary

a) Insurance is a 'promise' and the policy is a 'witness' to that promise. The occurrence of insured event leading to a claim under the policy is the true test of that promise.

b) One of the key rating parameter in insurance is the claims paying ability of the insurance company.

c) Customers, who buys insurance is the primary stakeholder as well as the receiver of the claim.

d) In Cashless claim a network hospital provides the medical services based on a pre-approval from the insurer/ TPA and later submits the documents for settlement of the claim.

e) In reimbursement claim, the customer pays the hospital from his own resources and then files claim with Insurer/ TPA for payment.

f) Claim intimation is the first instance of contact between the customer and the claims team.

g) If a fraud is suspected by insurance company in case of insurance claim, it is sent for investigation. Investigation of a claim could be done in-house by an insurer/ TPA or be entrusted to a professional investigation agency.

h) Reserving refers to the amount of provision made for all claims in the books of the insurer based on the status of the claims.

i) In case of a denial, the customer has the option, apart from the representation to the insurer, to approach the Insurance Ombudsman or the consumer Commissions or even the legal authorities.

j) Frauds occur mostly in hospitalization indemnity policies but Personal accident policies also are used to make fraud claims.

k) The TPA provides many important services to the insurer and gets remunerated in the form of fees.